



PROTOCOL

PACU Discharge Criteria

Scope (Staff):	Perioperative Clinical Staff
Scope (Area):	PACU, Operating Theatres

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Aim

To ensure postoperative patients remain in the Post Anaesthetic Care Unit (PACU) until they meet the required discharge criteria as assessed by the PACU nurse and ward nurse for discharge ward.^{2,4}

Risk

Serious potential consequences for postoperative patients if discharged to the ward area prior to the discharge criteria being met, including airway compromise, excessive sedation, inadequate incomplete pain or postoperative nausea and vomiting (PONV) management and incomplete ISOBAR handover.

Background

Unless otherwise requested by the Anaesthetist, the decision to discharge a patient from PACU will be made by the Registered Nurse caring for the patient (in conjunction with the PACU nursing coordinator, when that nurse is inexperienced in PACU nursing care). If any doubt exists regarding the patient's suitability for discharge, the Anaesthetist will be contacted to perform a clinical assessment.^{2,4}

Key Points

- A patient may be discharged from PACU once the discharge criteria have been met.
- Patients that do not meet the discharge criteria must be reviewed by an Anaesthetist.
- The Anaesthetist must be informed if the patient has triggered any PACU events on Theatre Management System (TMS), apart from those events that

are responsive to nurse initiations as per relevant policies, i.e. Temperature or PONV, prior to discharge from PACU.

- Paediatric Critical Care (PCC) and Neonatal Intensive Care Unit (NICU) patients will bypass PACU.
- High Dependency Unit (HDU) patients may be admitted to PACU at the direction of the Anaesthetist, as per the [Patient Transfer to HDU via PACU](#), they must meet the PACU discharge criteria as per ward patients.
- When a delay in transfer to the ward occurs due to bed availability, observations and nursing care will be provided until discharge by the PACU nurse.
 - Refer to the PCH [Postoperative Care procedure](#).
- Patients who have had procedures under local anaesthesia or sedation may return to the ward once they meet PACU discharge Criteria.
 - Refer to the PCH [Care of the Conscious Perioperative Patient](#) guideline.

Clinical Observations in PACU

- Patients having met discharge criteria will receive clinical observations with frequency dependent upon patient's clinical need. ³
- In the event of clinical deterioration, observations must be increased to a minimum of 2 minutely, in line with the [Resuscitation and Responding to Clinical Deterioration MET Review and Code Blue procedure](#).

Criteria for Discharge

- The patient must be easily rousable scoring ≤ 2 in accordance with the University of Michigan Sedation Score (UMSS). ¹
- The patient's observations are within normal limits for age taking into account pre-existing medical conditions.
- The PACU assessment score **must** be ≤ 4 .
- If the ward nurse is concerned that the patient may be at risk for developing complications during the transfer or upon immediate return to the ward, the PACU nurse will contact the appropriate anaesthetic / surgical team to request a review prior to discharge, whilst the ward nurse is present.
- The review must be documented in the patient's medical record by the attending surgeon or anaesthetist and recorded on TMS by the PACU nurse.
 - Refer to the PCH [Post Operative Care \(Nursing\)](#) guideline.

PARROT Assessment

- A full PARROT assessment must be conducted and recorded on the PARROT chart prior to discharge and the Early Warning Score (EWS) must be ≤ 3 .

- If any observations fall within a shaded area on the PARROT, the appropriate actions must be taken as per the EWS Escalation Pathway.
 - Refer to the PCH [Observation, Monitoring and Paediatric Acute Recognition and Response Observation Tool \(PARROT\)](#) procedure.
- A patient with an EWS above 3 must be reviewed by an anaesthetist. The patient may be discharged to the ward with an EWS higher than 3 with permission of the anaesthetist who must document instructions for modifications to the EWS.

Pain Management

- If PACU opioid protocol has been given, the patient must remain in PACU for a minimum of 20 minutes post the last opioid dose, and PACU discharge criteria must be met with a UMSS sedation score ≤ 2 .
- It is recommended that any patient requiring the following should stay in PACU for a minimum of 30 minutes post the last opioid dose:
 - greater than 5 doses of opioid protocol
 - greater than 3 doses of opioid protocol plus additional sedating agents / adjuvants.
 - Refer to the PCH [Intermittent Intravenous \(IV\) Opioid Analgesia in the Post Anaesthetic Care Unit \(PACU\)](#) protocol.
- An appropriate pain management plan has been initiated in accordance with Acute Pain Service (APS).
 - Refer to the PCH [Assessment of Acute Pain in Infants, Children, and Adolescents](#) guideline.
- For parenteral techniques, including intravenous and epidural, please ensure lines are connected and prescriptions correct and that this is confirmed with the ward staff collecting the patient during ISOBAR handover.
 - Refer to the following relevant guidelines:
 - [Opioid Infusion Management in General Wards Guideline](#)
 - [Ketamine Infusion Management in General Ward Areas Guideline](#)
 - [Intravenous Patient Controlled Analgesia \(PCA\) Guideline](#)
 - [Epidural Infusion Management Guideline](#)
- The PACU nurse must provide ward staff with detail regarding and reginal analgesia, and the expected duration of effect and importance of pressure area care with local anaesthetic use.
 - Refer to [Wound and Nerve Infusion Management Guideline](#)
 - Refer to [Pressure Injury Prevention and Management Guideline](#)

- Pain assessment is undertaken appropriate to the analgesia used and recorded on the Pain Assessment and Management Record (e.g. for epidurals - pain intensity score, FAS, sedation, dermatomes etc).

Management following administration of Naloxone and/or Flumazenil

- Following the administration of naloxone and / or flumazenil, the patient should remain in PACU for the following minimum times:

1 dose given	30 minutes
2 – 3 doses given	60 minutes
> 3 doses	120 minutes (may require PCC admission)

- The patient must remain in PACU until the PACU discharge criteria has been met with the UMSS sedation score ≤ 2 in addition to the above times.
- Both the treating anaesthetist (or Duty Anaesthetist after hours) and Acute Pain Service should be informed about the patient's condition prior to leaving PACU.
 - Refer to the PCH [Intermittent Intravenous \(IV\) Opioid Analgesia in the Post Anaesthetic Care Unit \(PACU\)](#) protocol.

Postoperative Nausea and Vomiting (PONV)

- For applicable patients, the PACU nurse must ensure PONV has been adequately controlled, appropriate anti-emetics prescribed on the paediatric National Inpatient Medication Chart (pNIMC), and given as per the [PONV Management in Children](#) guideline.

Patient Temperature

- The patient's temperature should be between 36°C and 37.5°C prior to discharge (considering any pre-existing condition and other patient factors).
 - Refer to the PCH [Postoperative Temperature Management - PACU Guideline](#).

Paediatric Anaesthesia Emergence Delirium (PAED) Scale Criteria

- Emergence delirium is a transient state of marked irritation and disassociation after the discontinuation of anaesthesia in some patients. It is more likely to occur in paediatric patients between 2 and 5 years old following short procedures using inhaled anaesthesia.

- A delirium assessment must be performed and documented on all patients once they are admitted to PACU and have a UMSS score of <2 every 10 minutes using the PAED scale, until the patient is discharged from PACU.
- A score ≥ 12 indicates a positive screen for delirium, and the PACU nurse must contact the treating anaesthetist or DA for a review.
- The PACU nurse must document the anaesthetic review and management plans on the Post Anaesthetic Care Unit Observation Chart MR844.04 and/or Digital Medical Record (DMR) and Theatre Management Systems (TMS).
- If a patient has a pre-existing cognitive impairment or development delay, the PAED scale may not be suitable. Document on PACU Observation Chart MR844.04 and/or DMR.
- A score of <12 on the PAED scale indicates that the patient does not have delirium and is meets the PACU discharge criteria for this component.

Paediatric Anaesthesia Emergence Delirium (PAED) Scale Criteria					
Criteria	Not at all	Just a little	Quite a bit	Very Much	Extremely
The child makes eye contact with parent / caregiver					
The child's actions are purposeful					
The child is aware of his / her surroundings					
The child is restless					
The child is inconsolable					
If PAED Score ≥ 12, an anaesthetist review is required					

Table 1 Paediatric Anaesthesia Emergence Delirium (PAED) Scale Criteria⁵

Intentionally retained surgical items

- Intentionally retained items, such as dental packs, and their removal in PACU must be clearly documented on the Surgical Count sheet.
 - Refer to the PCH [Surgical Count Policy](#).
- Following this, if dental packs are inserted in PACU, this needs to be recorded in the Additional Notes of the PACU Record, and the ISOBAR section needs to be completed with what is retained, the number retained, and signed off by both the PACU nurse handing over and the ward nurse accepting care.

HDU via PACU

- A HDU nurse will take bedside ISOBAR handover for the patient in PACU, and they will transport the patient.

- The anaesthetist will have called and already provided a verbal handover to the medical team in HDU.
 - Refer to [Patient Transfer to HDU via PACU](#)

Parental Presence

- PACU nurse will call for one parent / guardian to be brought to PACU once the patient has roused and maintaining own airway.
 - Refer to [Parental Presence in PACU Guideline](#)
- When in attendance immediate postoperative care and pain management to be explained to the patient and parent / guardian.

Documentation and Handover

- All documentation **must** be completed and clinical ISOBAR handover using the tool in the PACU Record provided to the ward nurse, who accepts responsibility of the patient.
 - Both PACU nurse and receiving nurse must sign the Read Back part of the record.
 - Refer to the [Clinical Handover - PCH Multidisciplinary](#) procedure.

Patient Equipment

- Patients with a tracheostomy tube will be transferred with the mandatory equipment requirements.
 - Refer to [PCH Tracheostomy Management Guideline](#) and [PCH Tracheostomy Care - PACU Procedure](#)
- Patients with a chest drain will be transferred with 2 tubing clamps available.
 - Refer to [PCH Chest Drain Management Procedure](#)

Related CAHS internal policies, procedures and guidelines
Assessment of Acute Pain in Infants, Children, and Adolescents (PCH Clinical Practice Manual)
Chest Drain Management (PCH Clinical Practice Manual)
Clinical Handover - PCH Multidisciplinary (PCH Clinical Practice Manual)
Epidural Infusion Management (PCH Medication Management Manual)
Intermittent Intravenous (IV) Opioid Analgesia in the Post Anaesthetic Care Unit (PACU) (PCH Perioperative Practice Manual)

Related CAHS internal policies, procedures and guidelines
Intravenous Patient Controlled Analgesia (PCA) (PCH Medication Management Manual)
Ketamine Infusion Management in General Ward Areas (PCH Medication Management Manual)
Observation, Monitoring and Paediatric Acute Recognition and Response Observation Tool (PARROT) (PCH Clinical Practice Manual)
Opioid Infusion Management in General Wards (PCH Medication Management Manual)
Parental Presence in PACU (PCH Perioperative Practice Manual)
Patient Transfer to HDU via PACU (Perioperative Practice Manual)
PONV Management in Children (PCH Perioperative Practice Manual)
Postoperative Care (PCH Clinical Practice Manual)
Postoperative Temperature Management - PACU (PCH Perioperative Practice Manual)
Pressure Injury Prevention and Management (PCH Clinical Practice Manual)
Resuscitation and Responding to Clinical Deterioration MET Review and Code Blue (PCH Clinical Practice Manual)
Tracheostomy Management (PCH Clinical Practice Manual)
Tracheostomy Care - PACU (PCH Perioperative Practice Manual)
Surgical Count (PCH Perioperative Practice Manual)

References
1. Malviya S, Voepel-Lewis T, et al. Depth of sedation in children undergoing computed tomography: validity and reliability of the University of Michigan Sedation Scale (UMSS). British Journal of Anaesthesia. 2002; 88(2): 241-245.
2. Australian College of Operating Room Nurses Ltd (ACORN). Standard: Surgical scrubbing, gowning and gloving. In ACORN standards for Nursing Role; Post Anaesthetic Recovery (PAR) Nurse. Including nursing roles, guidelines and position statements and competency standards, 2016. Adelaide, SA: Author.
3. Routine post anaesthetic observation (The Royal Children's Hospital Melbourne).

References

4. Position Statement on the Post-Anaesthesia Care Unit: Australian and New Zealand College of Anaesthetists (ANZCA), Page 8, PSO4 (A) 2020.
5. Sikich, J & Lerman, J., Development and psychometric evaluation of the pediatric anaesthesia emergence delirium scale. Anaesthesiology. 2004 100 (5) 1138-45.

This document can be made available in alternative formats on request.

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